

Report on Medico-Legal Malpractice, Institutional Negligence, and Jurisdictional Remedies: The Case of Involuntary Detention Following Traumatic Brain Injury in Northern Ireland

Executive Summary

This report constitutes an exhaustive forensic examination of the multi-layered systemic failures that precipitated the involuntary detention of the subject from September 2024 to January 2025 within the jurisdiction of Northern Ireland. The case presents a catastrophic convergence of clinical error—specifically the misdiagnosis of Traumatic Brain Injury (TBI) as a psychiatric disorder, known as "diagnostic overshadowing"—and institutional malfeasance by Queen's University Belfast (QUB) regarding the suppression of police-verified threat intelligence.

The analysis is structured to serve as a foundational document for high-court litigation initiated from London against Northern Ireland-based entities. It dissects the statutory framework of the **Mental Health (Northern Ireland) Order 1986**, establishing where procedural deviations occurred that render the detention unlawful.¹ It rigorously examines the duty of care owed by university administrators, particularly regarding the handling of **Police Service of Northern Ireland (PSNI)** threat messages (Osman warnings), and the culpability of senior figures such as Alistair Finlay in allegedly mishandling whistleblower evidence.³

Furthermore, the report provides a strategic roadmap for cross-jurisdictional litigation under the **Civil Jurisdiction and Judgments Act 1982**, addressing the complexities of suing a Northern Ireland domicile from England.⁵ It evaluates the specific torts of false imprisonment, clinical negligence, and misfeasance in public office, supported by an analysis of the "diagnostic overshadowing" phenomenon in emergency medicine.⁶ The findings indicate actionable breaches of the **Human Rights Act 1998** (Articles 2, 5, and 8) and significant deviations from **NICE Clinical Guidelines**⁸ and **GMC Good Medical Practice**.⁹

Part I: The Medico-Legal Framework of Involuntary Detention

The involuntary detention of an individual for a period exceeding four months implies a transition from emergency holding powers to a formal compulsory treatment regime. This section deconstructs the legislative machinery of the **Mental Health (Northern Ireland) Order 1986** (the "1986 Order") and contrasts the statutory requirements with the clinical reality of an undiagnosed Traumatic Brain Injury (TBI).

1.1 The Mental Health (Northern Ireland) Order 1986: Anatomy of a Wrongful Detention

Unlike the Mental Health Act 1983 which governs England and Wales, Northern Ireland operates under the 1986 Order. This legislation creates a distinct procedural landscape that must be navigated with precision. Any deviation from the prescribed forms or timelines renders the detention *ab initio* unlawful, grounding a claim for **false imprisonment**.

1.1.1 The Mechanics of Admission for Assessment (Article 4)

The initial detention likely commenced under **Article 4** of the 1986 Order, which provides for "Admission for Assessment." This is the statutory gateway through which the subject entered the psychiatric system.

- **The Application (Form 2):** In standard practice, an application for assessment is made by an **Approved Social Worker (ASW)** using Form 2.¹⁰ The ASW has a non-delegable statutory duty to interview the patient in a "suitable manner" and satisfy themselves that detention is the *most appropriate* means of care.¹¹ If the ASW relied solely on the "false reports" provided by QUB without conducting an independent, unbiased interview that considered the subject's account of a physical assault, they breached their statutory duty. The ASW is the "gatekeeper" designed to prevent medical tyranny; their failure to filter out a case of physical trauma represents a primary procedural failure.
- **The Medical Recommendation (Form 3):** This is the critical juncture of clinical and legal failure. Article 6 of the Order requires that the medical recommendation be signed by a registered medical practitioner who has personally examined the patient.¹
 - **The "Two-Day" Rule:** Crucially, the legislation mandates that this examination must have taken place "not more than two days before the date of the application".¹ This strict temporal limit is designed to ensure the detention is based on *current* clinical evidence, not historical hearsay.
 - **Grounds for Opinion:** The practitioner must state the grounds for their opinion that the patient suffers from a "mental disorder" warranting detention. If the Royal Victoria Hospital (RVH) doctors based their Form 3 recommendation on the "collateral history" provided by QUB—a history the user alleges was false—rather than on the objective signs of the concussion they failed to diagnose, the medical recommendation is factually flawed. A detention based on a recommendation that ignores the patient's primary presenting complaint (head injury) in favor of third-party hearsay is legally vulnerable.

1.1.2 The Transition to Article 12 (Detention for Treatment)

An Article 4 assessment order has a statutory lifespan of **14 days**.² The fact that the user was detained from September 2024 to January 2025—a period of approximately four months—indicates that the legal basis for detention shifted to **Article 12** (Detention for Treatment).

- **The Second Opinion Requirement:** To convert an assessment order into a treatment order, a *second* medical recommendation is required, typically from a psychiatrist on the staff of the hospital who has examined the patient. This implies that a second senior clinician also failed to diagnose the TBI, compounding the negligence.
- **The Rectification of Errors:** Article 11 allows for the rectification of minor administrative errors in the application forms within 14 days. However, a fundamental error—such as the misdiagnosis of a physical brain injury as a mental disorder—cannot be "rectified" administratively. It strikes at the heart of the validity of the detention.
- **Tribunal Oversight:** The detention's duration suggests that the case may have been reviewed by the **Mental Health Review Tribunal for Northern Ireland**. If the Tribunal upheld the detention, it suggests that the "diagnostic overshadowing" was systemic, with the Tribunal panel (comprising a lawyer, a medical member, and a lay member) failing to interrogate the etiology of the patient's symptoms.

1.2 Diagnostic Overshadowing: The Clinical Failure at Royal Victoria Hospital

The core of the medical malpractice claim lies in the phenomenon of **diagnostic overshadowing**. This is a documented cognitive bias where clinicians attribute physical symptoms to a patient's existing or presumed mental health label, thereby missing organic pathology.⁶

1.2.1 The Clinical Mimicry of Concussion and Psychosis

Traumatic Brain Injury (TBI), specifically concussion, presents with a constellation of neurocognitive and behavioral symptoms that can mimic acute psychiatric crisis. The failure of the Royal Victoria Hospital (RVH) to distinguish between the two is the crux of the **clinical negligence** claim.

- **Behavioral Dysregulation:** Concussion often results in damage to the prefrontal cortex, leading to disinhibition, aggression, irritability, and emotional lability.¹³ To an observer primed with a report of "mental instability" from QUB, these symptoms look like mania or agitation. To a neurologist, they look like a frontal lobe injury.
- **Cognitive Deficits:** Amnesia (anterograde and retrograde), confusion, "brain fog," and slowed processing speed are hallmarks of TBI.¹³ In a psychiatric context, these are often mislabeled as "thought disorder" or "dissociation."
- **Physical Signs:** Photophobia, nausea, and vestibular disturbance are specific to TBI. If the RVH staff ignored these physical complaints because they had already decided the patient was a "mental health patient," they breached the fundamental standard of care.

1.2.2 The Breach of NICE Guidelines (CG176)

The **National Institute for Health and Care Excellence (NICE)** guidelines are the gold standard for clinical practice in the UK, including Northern Ireland.⁸ **CG176 (Head Injury: Assessment and Early Management)** provides specific criteria for when a CT head scan is mandatory.

- **The Indication for Imaging:** NICE CG176 mandates a CT head scan for adults who have sustained a head injury and present with any of the following:
 - GCS < 13 on initial assessment.
 - GCS < 15 at 2 hours after injury.
 - Focal neurological deficit.
 - Post-traumatic seizure.
 - More than one episode of vomiting.¹⁵
- **The Negligence Argument:** If the user presented with a history of assault (blow to the head) and "confusion" (GCS < 15), a CT scan was mandatory. By failing to scan the patient, RVH staff deprived the user of the diagnostic test that would have proven the organic nature of their condition. This is a breach of the *Bolam* standard (no responsible body of doctors would fail to scan a confused head injury patient) and arguably the *Montgomery* standard (failure to inform the patient of the risks of not scanning).

1.2.3 The Feedback Loop of False Information

The snippet evidence suggests a robust data-sharing infrastructure between QUB and the **Belfast Health and Social Care Trust (BHSCT)**.¹⁶

- **Occupational Health Channels:** QUB has a contract with BHSCT for occupational health services.¹⁶ If this channel, or informal networks between university administration and hospital staff, was used to convey the "false report" that the student was mentally ill, it prejudiced the clinical assessment.
- **Collateral History Bias:** The **GMC Good Medical Practice** guidelines emphasize the importance of "collateral history" but also warn against bias.⁹ A psychiatrist's duty is to critically evaluate information from third parties. If the RVH psychiatrists accepted QUB's narrative without question, ignoring the patient's own report of assault and the physical signs of trauma, they failed in their professional duty to remain objective.

1.3 The "Phone First" Triage and Emergency Access

The Belfast Trust operates a "Phone First" triage system for head injuries and psychiatric presentations.¹⁹

- **The Triage Error:** If the user was triaged via this system, the initial categorization as "psychiatric" rather than "trauma" likely set the trajectory for the entire admission. This initial triage decision, if based on QUB's false report rather than clinical symptoms, represents the first link in the chain of causation for the wrongful detention.

Table 1: Comparative Symptomatology – The Basis for Misdiagnosis

Clinical Presentation	Interpretation via "Diagnostic Overshadowing"	Actual Etiology (Traumatic Brain Injury/Concussion)

	(Psychiatric)	
Aggression / Irritability	Acute Mania / Psychotic Agitation	Frontal Lobe Disinhibition: Damage to the orbital prefrontal cortex reduces impulse control.
Confusion / Disorientation	Delirium / Schizophrenia	Post-Traumatic Amnesia (PTA): Inability to lay down new memories due to hippocampal disruption.
Incoherence / Slurred Speech	Thought Disorder / Word Salad	Dysphasia / Dysarthria: Neurological deficit affecting speech motor planning or language processing.
Anxiety / Paranoia	Paranoid Delusions	Physiological Trauma Response: Hyperarousal of the amygdala due to the assault and ongoing threat.
Refusal of Admission	"Lack of Insight" (Capacity Issue)	Rational Fear: A logical response to being detained while suffering from an untreated physical injury.
Photophobia / Headache	Somatization / Attention Seeking	Post-Concussion Syndrome: Physical hypersensitivity typical of mTBI.

Part II: Institutional Liability of Queen's University Belfast

The culpability of Queen's University Belfast extends beyond mere administrative negligence into the realm of human rights violations. The University's refusal to act on a police-verified threat to life, leading to a second assault and subsequent detention, represents a profound failure of its safeguarding duties.

2.1 The *Osman* Warning: The Legal Duty to Protect Life

The "threat message" provided by the PSNI confirms the existence of an **Osman warning**. This legal concept, derived from the ECHR case *Osman v United Kingdom*, establishes that state authorities have a positive duty to take preventive operational measures to protect an individual whose life is at risk from the criminal acts of another.

2.1.1 Applicability to QUB as a Public Authority

While *Osman* duties traditionally bind the police, the **Human Rights Act 1998** (HRA) extends

these obligations to all "public authorities." As a university receiving public funding and exercising statutory powers, QUB is a "hybrid public authority".²⁰

- **Article 2 (Right to Life):** The HRA imposes a positive obligation on QUB to protect the life of a student when:
 1. **Knowledge:** The University knew of a "real and immediate risk" to the life of an identified individual (confirmed by the PSNI threat message).
 2. **Capacity:** The University had the power to take measures to avoid that risk (provision of emergency accommodation).
 3. **Failure:** The University failed to take those measures.
- **The Breach:** By refusing emergency accommodation in the face of verified police intelligence, QUB failed to discharge its Article 2 duty. This failure directly exposed the user to the "subsequent assault" mentioned in the query, which in turn contributed to the TBI and detention.

2.1.2 The "Consumer" Duty of Care

Beyond human rights, QUB owes a **common law duty of care** to its students. This is reinforced by the **Consumer Rights Act 2015**, which implies terms of "reasonable care and skill" into the student contract.²¹

- **Safeguarding Policy:** QUB's own **Safeguarding Children and Adults at Risk Policy** acknowledges a "statutory and moral duty" to safeguard the welfare of "at risk adults".²⁰ A student under a verified death threat is definitively an "at risk adult." The University's refusal to provide accommodation contradicts its own published policy, providing strong evidence of negligence.

2.2 Alistair Finlay: The Nexus of Intelligence and Administration

Alistair Finlay's role is pivotal. Identified in the research as a former **Assistant Chief Constable (ACC)** of the PSNI and currently the University Secretary/Chief People Officer at QUB³, his background fundamentally alters the legal analysis of QUB's liability.

2.2.1 Constructive Knowledge and "Willful Blindness"

- **Expertise:** As a former high-ranking police officer, Finlay possesses specialized knowledge regarding the severity of *Osman* warnings and threat management.²³ He cannot claim the defense of a lay administrator who "didn't understand" the police message.
- **Imputable Knowledge:** Finlay's knowledge of the threat is imputed to the University. If he reviewed the whistleblower evidence or the police threat and dismissed it, his actions may constitute **Misfeasance in Public Office**. This tort requires proving that a public officer acted with "targeted malice" or with "reckless indifference" to the probability that their act would injure the claimant. Ignoring a death threat fits the definition of reckless indifference.
- **Conflict of Interest:** Finlay's dual history (PSNI and QUB) raises questions about whether he prioritized the University's reputation over student safety. If he suppressed the threat

intelligence to avoid a "security incident" on campus, this is a distinct actionable wrong.

2.3 The Refusal of Emergency Accommodation

QUB's accommodation policies explicitly allow for **emergency accommodation**.²⁴

- **The "Guest" Provision:** The policy states that guests separated from residents may be offered emergency accommodation. It follows *a fortiori* that a registered student under a police threat should be afforded at least the same protection.
- **The "Safety Team" Authority:** The policies grant the Safety Team broad powers to exclude people for safety.²⁵ The converse implies a power to *include* or *house* people for safety. The refusal to exercise this power in an emergency is irrational and legally challengeable.

Part III: Whistleblowing and Administrative Justice

The user's attempt to "blow the whistle" on QUB's negligence was rejected. This rejection exposes a critical legislative gap in Northern Ireland law that the user must navigate carefully.

3.1 The "Worker" Definition Trap in PIDO 1998

The **Public Interest Disclosure (Northern Ireland) Order 1998** (PIDO) is the statutory framework for whistleblowing. However, it is designed primarily to protect *employees*.²⁶

- **Student Exclusion:** Snippets confirm that students are generally **not** considered "workers" under the 1998 Order unless they hold a specific employment contract (e.g., student nurses or PhD teaching assistants).²⁸ This is a major loophole.
- **The "Parkins v Sodexho" Loophole:** Although case law (*Parkins v Sodexho*) extended protection to breaches of individual contracts, the legislative intent remains focused on employment. QUB likely rejected the user's "whistleblower" submission on this technicality—that as a student, they had no standing to make a "protected disclosure" under the Order.
- **Strategic Pivot:** Instead of relying on PIDO, the user should frame the "whistleblowing" as a **Formal Student Complaint** regarding a breach of the **Student Charter** and **Safeguarding Policy**. While it lacks the statutory protection against "detriment" (retaliation), it triggers the internal review mechanisms necessary to exhaust remedies before approaching the Ombudsman.

3.2 The Northern Ireland Public Services Ombudsman (NIPSO)

For administrative failures (maladministration), the **NIPSO** is the court of last resort before judicial review.²⁹

- **Jurisdiction:** NIPSO investigates complaints against universities in Northern Ireland. Crucially, while NIPSO cannot investigate "academic judgment," the decision to refuse accommodation in response to a police threat is an **administrative** and **operational** decision, not an academic one. It falls squarely within NIPSO's remit.

- **Systemic Maladministration:** The user can argue that QUB's handling of the threat represents "systemic maladministration"—a failure of procedure that endangered a student. NIPSO has the power to recommend apologies, changes in procedure, and financial consolidation.
- **Time Limits:** A complaint to NIPSO must be made within **six months** of completing the University's internal complaints procedure.³¹

3.3 The "Fitness to Study" Weaponization

The snippets suggest that QUB may use "Fitness to Study" procedures to manage students with health issues.³²

- **The Mechanism:** It is highly probable that QUB initiated Fitness to Study proceedings against the user, citing the "mental health" reports (which the user claims are false) as the justification. This effectively bypasses the disciplinary process, allowing the University to suspend or remove the student on "medical" grounds without proving misconduct.
- **The Challenge:** This decision can be challenged if it was based on inaccurate medical evidence (the misdiagnosed TBI). The user must argue that the Fitness to Study decision was **irrational** because it ignored the competing evidence of the police threat and the concussion.

Part IV: Strategic Litigation from London

Litigating against Northern Ireland entities while resident in London involves complex questions of jurisdiction and procedure.

4.1 Jurisdiction: The Civil Jurisdiction and Judgments Act 1982

The **Civil Jurisdiction and Judgments Act 1982** (CJJA) governs the allocation of jurisdiction between the courts of the United Kingdom (England/Wales, Scotland, and Northern Ireland).

- **The General Rule (Schedule 4):** The basic rule is that a defendant must be sued in the courts of the part of the UK where they are **domiciled**.⁵
 - **Belfast Trust:** Domiciled in Northern Ireland.
 - **Queen's University Belfast:** Domiciled in Northern Ireland.
 - **Implication:** The user cannot simply file a claim in the High Court in London (Royal Courts of Justice). The claim **must** be issued in the **High Court of Justice in Northern Ireland** (Belfast).
- **Exceptions:** There are limited exceptions for contract (place of performance) or tort (place where the harmful event occurred). However, since the contract was for education in Belfast and the negligence/assault occurred in Belfast, the jurisdiction remains firmly in Northern Ireland.
- **Practicality:** The user does not need to *move* to Belfast. They can instruct **Northern Ireland-qualified solicitors** remotely. Many Belfast firms handle cases for clients in Great Britain.³⁴

4.2 Limitation Periods and the "Date of Knowledge"

- **Clinical Negligence:** Under the **Limitation (Northern Ireland) Order 1989**, the time limit is **three years** from the date of injury or the **date of knowledge**.³⁶
 - *Date of Knowledge:* Crucially, the clock likely did not start ticking in September 2024 (when the detention began) but rather in **January 2025** (or later), when the user discovered that the detention was based on a misdiagnosis of TBI. This preserves the limitation period.
- **Human Rights Claims:** Claims under the Human Rights Act 1998 (e.g., Article 2 Right to Life, Article 5 Liberty) have a much stricter deadline of **one year**.³⁶ The user must issue proceedings for these claims by **September 2025** (one year from the start of detention) or **January 2026** (one year from discharge). Immediate action is required.
- **Disability Extension:** If the user was legally "under a disability" (i.e., detained under the Mental Health Order) during the period, the limitation clock is paused until the disability ceases (discharge in Jan 2025).³⁶

4.3 Funding the Litigation

- **Legal Aid:** As a resident of England, the user is still eligible to apply for **Civil Legal Aid in Northern Ireland**, provided they meet the financial means test.³⁸ The application can be made via a solicitor in NI.
 - *Merits Test:* The user must show "reasonable grounds" for taking the case. The combination of the PSNI threat message and the medical misdiagnosis provides strong grounds.
- **After-the-Event (ATE) Insurance:** If Legal Aid is unavailable, the user should seek a "No Win, No Fee" solicitor (Conditional Fee Agreement). Essential to this is ATE insurance, which covers the user's liability for QUB's legal costs if the case is lost.³⁹

4.4 The Pre-Action Protocol

Northern Ireland strictly enforces a **Pre-Action Protocol for Clinical Negligence**.⁴⁰

1. **Request for Records:** The user's solicitor must formally request all GP, RVH, and QUB records.
2. **Letter of Claim:** A detailed letter must be sent to the Belfast Trust and QUB setting out the allegations. They have **four months** to investigate and respond.⁴¹

Conclusion

The user's situation represents a systemic failure of protection involving multiple statutory bodies. The detention appears to have been legally flawed due to the reliance on false hearsay over clinical examination (violating Article 6 of the 1986 Order). The misdiagnosis of TBI as psychiatric disorder constitutes clear *Bolam* negligence, exacerbated by the failure to follow NICE CG176 on head injury imaging. QUB's refusal to house a student under a police-verified

threat is a potential breach of Article 2 of the HRA.

Actionable Recommendations:

- 1. **Instruct NI Counsel:** Immediately engage a solicitor in Belfast specializing in medical negligence and human rights.
- 2. **Preserve Evidence:** Issue **Subject Access Requests (SARs)** to PSNI (for the threat log) and QUB (for Alistair Finlay's correspondence).
- 3. **Medical Expert Report:** Commission a retrospective report from a Consultant Neurologist to validate the TBI diagnosis.
- 4. **File NIPSO Complaint:** Lodge a complaint regarding "maladministration" for the refusal of emergency accommodation.

Table 2: Legal Limitation Deadlines (Northern Ireland)

Cause of Action	Relevant Statute	Time Limit	Critical Date (Start of Clock)
Clinical Negligence	Limitation (NI) Order 1989	3 Years	Date of knowledge (discovery of misdiagnosis)
Personal Injury	Limitation (NI) Order 1989	3 Years	Date of assault/injury (Sept 2024)
Human Rights Breach (Art 2/5)	Human Rights Act 1998	1 Year	Date of the breach (detention start or accommodation refusal)
Judicial Review	RSC (NI) Order 53	3 Months	Date of the specific decision challenged
False Imprisonment	Limitation (NI) Order 1989	6 Years	Date of release from detention (Jan 2025)

Table 3: Key Actors and Potential Liability

Actor	Role	Potential Liability	Key Evidence Needed
Belfast Health & Social Care Trust (RVH)	Clinical Provider	Clinical Negligence: Failure to diagnose TBI (NICE CG176 breach). False Imprisonment: Unlawful detention under MH Order.	ED Triage Notes, CT Scan records (or lack thereof), Form 3 Medical Recommendation.
Queen's University Belfast (QUB)	Educational Institution	Negligence: Breach of Duty of Care (Safeguarding).	Accommodation refusal emails, internal memos re: PSNI threat.

		Human Rights Act: Breach of Article 2 (Right to Life).	
Alistair Finlay	University Secretary (Ex-PSNI)	Misfeasance in Public Office: Willful disregard of police intelligence. Administrative Negligence: Mishandling of whistleblower report.	Internal emails, meeting minutes, whistleblower report logs.
PSNI	Police Service	Operational Duty Breach: Failure to enforce protection if QUB refused.	Threat Message Log (Osman Warning), Incident Reports.

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